

Scene Safety & Assessment

#1

DISPATCH:
Your hiking group hears a scream from the next campsite. You arrive to find a 34-year-old male lying on the ground next to an overturned camp stove. Flames are nearby.

CHIEF COMPLAINT:
I slipped and burned my arm!

MECHANISM:
Fell onto burning camp stove, superficial burns to right forearm, stove still has small flame

VITALS:
HR 98 | RR 18 | Skin warm/moist | LOC Alert x 4 | BP normal

INTERVENTIONS:
Scene size-up first (fire hazard)

TIP: Students often rush past scene safety. The bu

Complete Patient Assessment

#2

DISPATCH:
A 45-year-old female fell while descending a rocky trail. She is sitting up and holding her wrist.

CHIEF COMPLAINT:
I tripped and caught myself — my wrist is really sore.

MECHANISM:
Trip and fall, outstretched hand, heard a pop

VITALS:
HR 88 | RR 16 | Skin pink/warm/dry | LOC Alert x 4

INTERVENTIONS:
Full XABCDE

TIP: Complete full systematic assessment even for

Airway Obstruction#3

DISPATCH:
During lunch at the trailhead, a 52-year-old male suddenly stands up, clutches his throat, and cannot speak or cough effectively. His face is reddening.

CHIEF COMPLAINT:
(Patient cannot speak — gestures to throat)

MECHANISM:
Eating quickly while talking, sudden complete airway obstruction

VITALS:
HR rapid | RR 0 effective | Skin flushing then cyanotic | LOC Alert → declining

INTERVENTIONS:
Recognize obstruction (universal choking sign)

TIP: Speed matters — brain damage in 4-6 min. Don’

Severe Hemorrhage#4

DISPATCH:
A 29-year-old male fell while crossing a river and his leg was lacerated on a sharp rock. He has dragged himself to shore. Large pool of blood around right thigh.

CHIEF COMPLAINT:
I can't stop the bleeding — it's pumping out!

MECHANISM:
Deep laceration from sharp rock, arterial hemorrhage from right thigh

VITALS:
HR 118 rapid/weak | RR 22 | Skin pale/cool/moist | LOC Alert but anxious | BP low

INTERVENTIONS:
PPE first

TIP: This simulates arterial bleeding. Escalate to

Shock

#5

DISPATCH:

A 38-year-old female has been found in her tent mid-morning. Her tentmate reports she had severe diarrhea and vomiting throughout the night and just doesn't look right.

CHIEF COMPLAINT:

I feel dizzy and really weak... I can barely sit up.

MECHANISM:

12+ hours of vomiting and diarrhea, suspected severe dehydration/hypovolemic shock

VITALS:

HR 126 weak | RR 24 | Skin pale/cool/moist | LOC confused | Cap refill 4 sec

INTERVENTIONS:

XABCDE (shock recognition)

TIP:

Non-hemorrhagic hypovolemic shock is common.

Femur Fracture

#6

DISPATCH:

A 44-year-old male slipped off a boulder and landed hard on his right hip. He cannot bear weight and his right leg appears shortened and externally rotated.

CHIEF COMPLAINT:

Something snapped — I can't move my leg at all.

MECHANISM:

Fall from 6 feet onto right hip, suspected femur fracture with significant deformity

VITALS:

HR 120 | RR 20 | Skin pale/cool | LOC Alert but severe pain

INTERVENTIONS:

XABCDE (shock protocol)

TIP:

Femur fractures = treat for shock immediately

Spine Injury

#7

DISPATCH:

A 22-year-old male dived into a lake from a cliff and struck the bottom. He is floating face-down in shallow water when bystanders pull him out. He is conscious but confused.

CHIEF COMPLAINT:

My neck really hurts... I can't feel my feet.

MECHANISM:

Axial loading — diving into shallow water, suspected cervical spine injury

VITALS:

HR 68 (bradycardia — neurogenic shock) | RR 18 | Skin warm | LOC Verbal

INTERVENTIONS:

In-line stabilization immediately

TIP: Bradycardia + hypotension after spinal injury

Traumatic Brain Injury

#8

DISPATCH:

A 31-year-old female was struck by a falling branch while hiking. She was briefly unconscious and is now awake but confused and keeps asking what happened?

CHIEF COMPLAINT:

I have a terrible headache and I feel sick to my stomach.

MECHANISM:

Strike to head by falling branch, brief LOC, now GCS 13 (will worsen during scenario)

VITALS:

HR 58 (Cushing's triad early) | RR 14 | Skin pink | LOC Confused — deteriorating

INTERVENTIONS:

In-line stabilization (trauma mechanism)

TIP: Falling GCS after TBI = herniation risk. Reas

Heat Stroke

#9

DISPATCH:
A marathon trail runner is found staggering off the trail on a hot summer day. A bystander says she was running fine but suddenly started weaving and fell.

CHIEF COMPLAINT:
(Patient cannot give coherent history — confused, combative)

MECHANISM:
Exertional heat stroke, ambient temp 95°F, exercising 4 hours without adequate hydration

VITALS:
HR 142 | RR 26 | Skin hot and moist | Temp 105°F | LOC Confused/combative

INTERVENTIONS:
Recognize heat stroke (altered LOC = key)

TIP: Cool first, transport second. Students skip c

Hypothermia

#10

DISPATCH:
A 55-year-old male got separated from his group during a rainstorm last night. He is found in the morning sitting against a tree, unresponsive to voice but groaning when touched. His clothes are soaking wet.

CHIEF COMPLAINT:
(Patient only responds to pain — groans)

MECHANISM:
Overnight exposure to rain, suspected moderate-to-severe hypothermia

VITALS:
HR 38 irregular/very slow | RR 8 shallow | Skin cold/pale/mottled | LOC Pain (P on AVPU)

INTERVENTIONS:
Handle extremely gently (V-fib risk)

TIP: Hypothermic until warm and dead. Handle like

Frostbite

#11

DISPATCH:

A 40-year-old skier reports that three toes on her left foot went numb and white three hours ago during a storm. She has been warming them in her boot since but they still feel strange.

CHIEF COMPLAINT:

My toes are white and hard — they don't hurt anymore, which scares me.

MECHANISM:

Prolonged cold exposure in wet ski boot, deep frostbite to three toes

VITALS:

HR 82 | RR 16 | Skin otherwise normal | LOC Alert × 4

INTERVENTIONS:

Do NOT rewarm (re-freeze risk on 8-mile ski out)

TIP: Key decision: will they rewarm? No — re-freez

Anaphylaxis

#12

DISPATCH:

A 27-year-old female was stung by bees (3 stings) 10 minutes ago while hiking. A bystander says she has a bee allergy. She is sitting upright, breathing hard with audible wheezing.

CHIEF COMPLAINT:

I can't breathe... my throat is tight... I have an EpiPen in my pack.

MECHANISM:

Bee sting anaphylaxis in known allergic patient

VITALS:

HR 132 | RR 28 with audible wheeze | Skin flushing with hives | LOC Alert but very anxious

INTERVENTIONS:

Epinephrine IMMEDIATELY (help patient with EpiPen — outer thigh)

TIP: Students want antihistamines first. Epi FIRST

Diabetic Emergency

#13

DISPATCH:

A 34-year-old male who is a known insulin-dependent diabetic is found sitting on a log, staring blankly. His hiking partners say he ate very little at breakfast and has been acting weird for 20 minutes.

CHIEF COMPLAINT:

(Patient confused, making little sense — shaking and diaphoretic)

MECHANISM:

Insulin-dependent diabetic who skipped breakfast — likely hypoglycemia

VITALS:

HR 112 | RR 18 | Skin pale/diaphoretic | LOC
Confused — responds to voice

INTERVENTIONS:

Recognize hypoglycemia (rapid onset, sweating, confusion in diabetic)

TIP: Rapid onset + sweating + diabetic = hypoglyce

Seizure

#14

DISPATCH:

A 19-year-old college student on a backpacking trip begins shaking uncontrollably. Her companions say she has no history of seizures and was normal moments before.

CHIEF COMPLAINT:

(Postictal — confused, not responsive to questions, no memory of event)

MECHANISM:

New-onset generalized tonic-clonic seizure, approximately 90 seconds duration

VITALS:

POST-SEIZURE: HR 130 | RR 22 | Skin flushed/slightly cyanotic lips | LOC Verbal (postictal)

INTERVENTIONS:

During: protect from injury (clear area), do NOT restrain, nothing in mouth, time seizure

TIP: Students try to restrain or put something in

Stroke

#15

DISPATCH:

A 67-year-old female on a guided hike suddenly stops and grabs a tree for support. She tells her guide, I feel very strange. Her speech is slurred.

CHIEF COMPLAINT:

My arm feels heavy... I can't get my words out right.

MECHANISM:

Sudden neurological event in older adult, suspected ischemic stroke

VITALS:

HR 88 irregular (atrial fibrillation) | RR 16 | Skin normal | LOC Alert but confused

INTERVENTIONS:

Recognize stroke (FAST)

TIP: Time is brain — every minute = 2 million neur

Snakebite

#16

DISPATCH:

A 24-year-old male reached under a rock while scrambling and was struck twice on the right hand. He reports seeing a thick brownish snake before it slid away. His hand is swelling rapidly.

CHIEF COMPLAINT:

It bit me twice — my hand is on fire and it's getting huge!

MECHANISM:

Pit viper envenomation (likely rattlesnake), two bites to dorsum of right hand

VITALS:

HR 96 | RR 18 | Skin normal (systemic symptoms developing) | LOC Alert

INTERVENTIONS:

Remove rings/watch on affected hand (swelling)

TIP: Most common errors: cutting & sucking, tourni

Open Fracture with Wound

#17

DISPATCH:

A 48-year-old male mountain biker crashed at high speed and the bike frame penetrated his lower left leg. There is an obvious deformity with bone visible through the wound.

CHIEF COMPLAINT:

I can see bone. This is really bad.

MECHANISM:

High-energy trauma, open (compound) tibia fracture

VITALS:

HR 108 | RR 20 | Skin pale/cool | LOC Alert but severe pain and near syncope

INTERVENTIONS:

PPE

TIP: Open fractures have high infection risk. Stud

Multiple Patient Scene

#18

DISPATCH:

A van carrying a hiking group has rolled off the road. There are 5 patients. You arrive first on scene with two other WFA providers.

CHIEF COMPLAINT:

Multiple patients: P1 unconscious | P2 deformed leg | P3 walking screaming | P4 facial lacerations | P5 sitting holding chest

MECHANISM:

Motor vehicle rollover, multiple mechanisms of injury. Mass casualty triage scenario.

VITALS:

TRIAGE SETUP: P1 unconscious, P2 deformed leg, P3 ambulating, P4 alert, P5 chest pain

INTERVENTIONS:

Scene safety first

TIP: Don't get fixated on one patient. The patient

Improvised Litter Carry

#19

DISPATCH:

A 58-year-old female slipped on wet rock and sustained a suspected pelvis fracture. She is in severe pain and cannot bear any weight. The trailhead is 2 miles away via smooth trail.

CHIEF COMPLAINT:

I cannot move my hips without incredible pain — please don't move me.

MECHANISM:

Slip and fall, axial compression, severe pelvis pain with any movement

VITALS:

HR 120 | RR 22 | Skin pale/cool | LOC Alert

INTERVENTIONS:

Spine precautions (pelvis injury + mechanism)

TIP: Pelvic fractures can cause 3+ liters blood lo

Capstone Scenario

#20

DISPATCH:

6 miles from trailhead, your group hears an explosion from the river below. A kayaker pinned by a log has been freed by other paddlers. He is conscious but pale. 38°F and drizzling.

CHIEF COMPLAINT:

I was under for a long time... I can't feel my left leg well.

MECHANISM:

Hydraulic entrapment, possible near-drowning, cold water immersion, suspected spinal and extremity injuries

VITALS:

HR 88 irregular | RR 22 shallow | Skin cold/pale/wet | LOC Alert but confused — declining during scenario

INTERVENTIONS:

Scene safety (unstable riverbank)

TIP: Capstone tests everything: scene safety, mult